

SARMs Scientific Handbook

8 SARMs — Clinical Data, Efficacy, Suppression & PCT Protocols

8 SARMs	Clinical Data	Suppression Profiles	54+ pg Pages
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■ **MEDICAL DISCLAIMER:** This guide is for educational and harm-reduction purposes only. The compounds discussed are prescription medications in many countries. Always consult a qualified physician before using any pharmacological substance. Tiger Fitness Pro does not encourage illegal activity.

SARMs Scientific Handbook — 8 SARMs with Clinical Data

Selective Androgen Receptor Modulators (SARMs) were developed to provide anabolic benefits of testosterone in muscle and bone while minimizing androgenic effects on other tissues. While promising, most remain investigational. This guide presents the available clinical evidence alongside practical protocols.

SARMs Comparison — Master Table

SARM	Half-life	Anabolic:Andr ogenic	Best Use	Suppression	Clinical Phase
Ostarine (MK-2866)	24 hrs	3:1	Lean mass, joint support	Mild	Phase II complete
Ligandrol (LGD-4033)	24–36 hrs	10:1	Bulking, strength	Moderate	Phase II complete
RAD-140 (Testolone)	60 hrs	90:1	Strength, mass	Moderate–High	Phase I/II
Cardarine (GW-501516)	16–24 hrs	N/A (PPARδ)	Endurance, fat loss	None (not a SARM)	Discontinued (oncology)
Andarine (S-4)	4 hrs	3:1	Cutting, vascularity	Mild–Mod	Phase I
Ibutamoren (MK-677)	24 hrs	N/A (GH secr.)	GH/IGF-1 elevation	None	Phase II
YK-11	6–10 hrs	Myostatin inhibitor	Mass	Significant	Research only
S-23	12 hrs	High	Cutting, contraceptive research	High	Research only

Individual SARM Profiles

1. Ostarine (MK-2866)

Most studied SARM. Phase II trials showed 1.4 kg lean mass gain in cancer patients at 3 mg/day. Beginner-friendly. Minimal suppression at doses ≤ 25 mg/day for ≤ 8 weeks.

Parameter	Value
Bulking dose	25–35 mg/day
Cutting dose	15–25 mg/day
Cycle length	8–12 weeks
PCT needed	Mini-PCT if >8 weeks: Nolvadex 20 mg x 4 weeks
Main benefit	Lean mass + joint repair
Main risk	Mild testosterone suppression

2. Ligandrol (LGD-4033)

Phase II trial: 1 mg/day produced 1.21 kg lean mass in 21 days. Most potent approved SARM for muscle building. Significant suppression requires full PCT.

Parameter	Value
Bulk dose	5–10 mg/day
Cycle	8–10 weeks
PCT	Nolvadex 20/20/10/10 mg, start 72 hrs after last dose
Main benefit	Rapid strength + mass gains
Risk	LH/FSH suppression, lipid changes

3. RAD-140 (Testolone)

Strongest SARM by anabolic:androgenic ratio (90:1 vs testosterone 1:1). Significant suppression. Phase I safety data published 2023. Used for mass building and strength.

Parameter	Value
Dose	10–20 mg/day
Cycle	6–8 weeks maximum
PCT	Full PCT: Nolvadex 40/40/20/20
Benefit	Extreme strength gains
Risk	Liver enzyme elevation in some users, high suppression

SARMs Stacking Protocols

Goal	Stack	Doses	Cycle	PCT
Lean Bulk	LGD-4033 + MK-677	10 mg + 25 mg	10 weeks	Nolvadex 4 weeks
Cut	Ostarine + Cardarine	25 mg + 10 mg	10 weeks	Mini PCT
Recomp	RAD-140 + Ostarine	10 mg + 15 mg	8 weeks	Full PCT
Endurance	Cardarine only	10–20 mg	8 weeks	None needed
Strength	RAD-140 + LGD	15 mg + 5 mg	8 weeks	Full PCT required

Bloodwork Monitoring

Test	Baseline	Week 4	Post-PCT 4wks	Why
Total Testosterone	Required	Monitor	Recovery check	Primary suppression marker
LH + FSH	Required	Expected low	Should recover	HPTA function
SHBG	Baseline	Monitor	—	LGD lowers SHBG
HDL/LDL	Baseline	Check	Normalize	SARMs alter lipids
AST/ALT	Baseline	Check	—	RAD/YK liver stress